BASELINE → TREATMENT → FOLLOW-UP

Ribociclib: checks before starting and within the early cycles

Days 1–21: ribociclib. Days 22–28: break. The endocrine partner follows its own schedule. Day 25–26 is a local operational option, not an NCCP replacement for early checks.

Check	Before first dose	Cycle 1 Day 14	Cycle 2 · start and Day 14	Cycles 3–6 · before each	Later cycles
Blood count / liver / renal	Baseline	Two-week check	Continue two-weekly checks	Before each cycle	As clinically indicated; agree the local plan
ECG · heart rhythm	Required; assess QTcF	Repeat around Day 14	If clinically indicated	If clinically indicated	If symptoms, QT concerns or interacting medicines
Electrolytes	Baseline; correct abnormalities	Additional tests if indicated	At start of cycle	At start of each cycle	As clinically indicated
Restart review	Confirm prescription and interactions	Review results; hold if required	No automatic restart	Optional local D25–26 review window	Team confirms each hold / restart plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00525 v6 · official regimen · Novartis · Kisqali product information, updated September 2026

THE WHOLE TREATMENT JOURNEY

Ribociclib: tablets, early checks and response reviews

Advanced / metastatic · HR-positive / HER2-negative breast cancer

Ribociclib is taken for three weeks of each four-week cycle with endocrine treatment. The first two cycles need extra blood checks and an early ECG; cancer-response reviews continue over the longer term.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycle 1 · weeks 1–4	Cycle 2 · weeks 5–8	Cycles 3–6	After cycle 6
Ribociclib NCCP 00525	Prescription and interaction review	Days 1–21 tablets; days 22–28 off	Repeat 21 days on / 7 off if cleared	Repeat the same cycle unless changed	Continue while benefit and tolerance support it
Endocrine treatment NCCP 00525	Choose AI OR fulvestrant; ovarian suppression if needed	AI daily throughout OR fulvestrant loading visits	AI continues during ribociclib week off; injections as planned	Continue endocrine partner	Continue agreed endocrine plan
Blood tests NCCP 00525	Blood count, kidney/liver tests; electrolytes	Bloods every 2 weeks	Bloods every 2 weeks	Before each cycle	As clinically indicated
ECG / electrolytes NCCP 00525 · RIBO	ECG; correct low potassium, magnesium, calcium or phosphate	ECG around Day 14; electrolytes before cycle	Electrolytes before cycle; ECG if indicated	Electrolytes before each cycle through cycle 6	ECG/electrolytes as indicated
Cancer scans / markers METASTATIC · MARKERS	Baseline imaging of known disease	Symptoms reviewed between scans	Arrange the first response review	Usually scan review every 2–4 months	Continue individualised response reviews
Bone / local care BONE · NOGG · METASTATIC	Assess bone risk with AI/OFS	DEXA or bone medicine if indicated	Radiation only under a separate local-treatment plan	Repeat bone assessment by risk	Continue endocrine and skeletal-health support

What each check is for

1 ECG: an electrical heart tracing

Before starting and around Day 14 of cycle 1; then as indicated

Ribociclib can alter the QT interval, which affects heart rhythm.

Ask / act: Attend the early ECG and report fainting, palpitations or new dizziness promptly.

Sources: NCCP 00525 · RIBO

2 Bloods and electrolytes

Bloods every 2 weeks for cycles 1–2; before cycles 3–6; then as indicated

Checks blood counts, kidney/liver tests and minerals relevant to heart rhythm.

Ask / act: Ask who checks the results before the next pack starts.

Sources: NCCP 00525

3 Cancer-response imaging

Usually every 2–4 months, adapted to the clinical picture

A baseline scan records where the cancer is. Response is usually reviewed every 2–4 months, adjusted for the treatment, disease pace, symptoms and previous results. CT chest/abdomen/pelvis is common when appropriate; the team chooses the imaging needed for the known sites.

Ask / act: Ask how scan results, symptoms and your priorities will guide the next decision.

Sources: METASTATIC

4 Markers, if selected

Alongside the individual response plan

CA 15-3 or CEA may be helpful in selected metastatic cases, especially if previously raised. They support the assessment with symptoms and imaging; a marker alone does not decide whether treatment is working or should change.

Ask / act: Ask whether markers add useful information in your case.

Sources: MARKERS

5

Bone health with endocrine therapy

At AI/OFS initiation and then by risk

Assess fracture risk and whether a baseline DEXA bone-density scan is needed when starting an aromatase inhibitor or ovarian suppression/treatment-induced menopause. It may already have been done before this regimen.

Ask / act: Ask about DEXA, weight-bearing activity and any indicated bone treatment.

Sources: BONE · NOGG

6

Week off ribociclib

Days 22–28 of each cycle

The planned break applies to ribociclib, not automatically to the endocrine partner.

Ask / act: Follow the written prescription for each medicine; do not make up missed or vomited doses.

Sources: NCCP 00525 · RIBO

7

Optional pre-cycle review window

Local Day 25–26 if adopted

Allows time for results before the planned next cycle.

Ask / act: Restart only after the team confirms the result and prescription.

Sources: NCCP 00525

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.